



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ورده عبدالودود عبدالمحسن عبدالعاطي
المحافظة : القاهرة الحي / المدينة : بولاق ابو العلا

العنوان : --

تاريخ تسجيل الإستمارة : 20-09-2026

رقم البطاقة : 27702072500608 رقم التليفون : 01007371565

العمر (تاريخ الميلاد) : 07-02-1977 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة



الاسم : د/محمد كيلاني الوظيفة : التوقيع :

الاسم : د/ندى حسن الوظيفة : التوقيع :

الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/ د/ د/

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **ورده عبدالودود عبدالمحسن عبدالعالم**
National ID: **27702072500608**



تقرير طبي

المسيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المريضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound

الوقت :

---:---

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة حقوق الانتخابية



ورده

عبدالودود عبدالمحسن عبدالعاطي

١٩ ش احمد علي اسماعيل

بولاق الدكرور - الجيزة



٢٧٧ . ٢ . ٧ ٢٥ . ٠ ٦ . ٠ ٨

K03124675



٢٠٢٤/٠٥ ٢٧٧٠٢٠٧٢٥٠٠٦٠٨

رخصة منزل



أنتى مسلمة متروجة

أحمد عبدالوهاب محمد عبدالعاطي

البطاقة سارية حتى ٢٠٣١/٠٥/١٩





التاريخ: ٢٠٢٦/٧/٢٨

الاسم: وردة عبد الوادود

Ultrasonography of the Abdomen

Liver :
Size ☒ Normal ☐ Small ☐ Enlarged (☐ Mild - ☐ Moderate ☐ Marked)
Surface ☒ Smooth ☐ Irregular
Texture ☒ Normal ☐ Coarse ☐ Heterogeneous ☒ Bright
P.V. ☒ Normal ☐ Dilated
Biliary radicles ☒ Normal ☐ Dilated CBD ☐ Normal ☐ Dilated
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Focal Lesions ☒ No ☐ Others

Gall Bladder :

Size ☒ Average ☐ Contracted ☐ Distended ☐ Removed
Wall Thickness : ☒ Normal ☐ Thickened
Mud ☒ No ☐
Stones ☒ No ☐

Spleen :

Size ☒ Average ☐ Enlarged (☐ Mild - ☐ Moderate ☐ Marked)
Echopattern : ☒ Normal ☐

Kidneys :

	Rt.	Lt.
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Parenchymal echopattern :	Simple cortical	
Focal lesion :	Cyst 33 mm	WAD
Stones :		
Back pressure :		

Pancreas : ☒ Normal ☐
Ascites : ☒ Non ☐ Mild ☐ Moderate ☐ Marked
Aorta and para aortic nodes : Normal ☒
Urinary bladder ☒ No stones ☐
Prostate ☒ Normal ☐ Enlarged (vol. = cm³)

Impression : Bulky uterus with post. wall fibroid
47 x 37 mm

Signature

Dr. El...

fatty liver
Rt Renal Simple Cyst
uterine fibroid



Name: Warda 3Abdelwadoud 45 Y, female

Date : 11/8/2026

PelviabdominalULTRASOUND

-liver: size in MCL : 13.2cm , texture : bright, surface : smooth , hepatic veins : normal , PV: not dilated , IHBRs : not dilated , signs of fibrosis or cirrhosis : no , F.Ls: no

-GB: size : average, shadow inside : no , wall thickness : thin , CBD : not dilated.

Pancreatic area : average size , free .

-Spleen : long axis :8.9cm , texture : homogenous , SP.V and collaterals : not dilated

-Kidneys : both are of normal position , texture : normal , back pressure : no , cysts : no , solid masses : no , stones : sandy , size : [11.6x4.7 cm , 11.3x4.3cm] .

-Pelvis:-UB: many internal echoes

- uterus & adenexa : a big heterogenous mass ? uterine versus adenexal : 5.1x5.0x5.2 cm for gynecological exam

-Ascites : no, pleural effusion : no

*Conclusion :

.Bright liver ? fatty , an umbilical hernia with defect : 1.4 cm , perumbilical one with defect : 5.2 cm .

.Bilateral renal sandy stones .

.severe Colonic distension , pelvis as above .

BEST REGARDS
MANAL SHAKER



اسم المريض : ورده عبد الودود عبد المحسن عبد العا
محول من :
تاريخ الحضور : 2026/07/07
تاريخ الاستلام : 2026/07/20
السن : 49 سنة
(2026) (1743 - 2026)

NATURE OF SPECIMEN :

Left breast mass, tru-cut biopsy.
Left axillary pathological lymph nodes, tru-cut biopsy

GROSS :

Two specimens are received designated as:

- 1) Left breast: Multiple gray white tissue cores the longest measured 1.5cm long, totally submitted.
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MICROSCOPY :

- 1) Sections examined revealed mammary tissue cores showing sheets, and clusters of malignant epithelial cells infiltrating among desmoplastic stroma. Tumor cells show moderate pleomorphism and moderate tubule formation. Intermediate grade DCIS, 10% (Solid pattern) Positive lymphovascular emboli.
- 2) Sections examined revealed lymph nodal tissue infiltrated by the same tumor seen in the breast.

DIAGNOSIS :

- 1) Left breast mass, tru-cut biopsy;
Invasive duct carcinoma, provisional core grade II.
Intermediate grade DCIS, 10% (Solid pattern)
Positive lymphovascular emboli.
- 2) Left axillary lymph nodes, tru-cut biopsy;
Metastatic Invasive duct carcinoma to the lymph node.

RECOMENDATION :

Prof. Dr. Rasha Farouk, MD.

رقم البطاقة: 27702072500608

The General Organization for Teaching
Hospitals and Institutes
Ahmed Maher Teaching Hospital



بنة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

اسم المريض : وردة عبد الودود عبد المحسن عبد العا
محول من :
تاريخ الحصول : 2026/07/20

السن : 49 سنة
(1743 - 2026)
تاريخ الإسلام : 2026/08/05

NATURE OF SPECIMEN :

One paraffin block (Left breast mass, tru-cut biopsy)

GROSS :

One paraffin block: sections immunostained for ER, PR, Her-2/neu, ki67, using supersensitive monoclonal antibodies with En- Vision TM+ (HRP) detection system employing peroxidase after antigen retrieval and blocking internal peroxidase and using DAB as chromogen using BenchMark GX automated immunostainer

MICROSCOPY :

Estrogen receptors: Strong nuclear staining in 90 % of tumor cells.
Progesterone receptors: Moderate nuclear staining in 50% of tumor cells.
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NULL HER-2/neu, SCORE 0
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RECOMMENDATION :

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قسم الباثولوجي

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(2026) (1743 - 2026)

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اسم المريض : ورده عبد الودود عبد المحسن عبد العال

محول من :

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RECOMMENDATION :

Prof. Dr. Rasha Farouk, MD.

رقم البطاقة: 27702072500608

Patient Name: *Warda Abdel Wadod Abdel-Mahssen*
Date: 1/7/2026

FULL FIELD DIGITAL MAMMOGRAPHY COMPLEMENTARY ULTRASOUND

Indication:

- Left axillary palpable lump.

Technique:

- CC and MLO views.
- High resolution ultrasound.

Findings:

- Scattered fibroglandular parenchyma. TAC of
- Left breast LOQ area of asymmetry.
- Left breast scattered macrocalcifications.
- No spiculated masses or grouped micro-calcifications.
- Complementary ultrasound showed both breasts are of homogeneous texture.
- Left breast LOQ at 3-4 O'clock area of altered parenchyma.
- No internal vascularity on color Doppler.
- Left breast para-areolar at 6 O'clock single tubular duct ectasia with nuclear echogenic non-vascular content.
- No cystic or solid masses are detected.
- Normal skin thickness & nipple/areola contour.
- Right Non specific axillary lymph nodes.
- Left axillary pathological lymph nodes have thin capsule.
- Right axillary pathological lymph nodes have thin capsule.
- Right largest measures 4x3.5 cm with irregular margins.
- Level II&III pathological lymph nodes.
- No evidence of supraclavicular lymph nodes.

Conclusion:

- Left breast area of altered parenchyma with cystic duct ectasia.
- Lymph nodes for histopathological assessment. (BIRADS 4C).
- Left breast ductectasia.
- Right breast normal study (BIRADS 1).

Recommendations:

- Fine cut biopsy of left breast altered parenchyma and lymph nodes.

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Ahmad Maher Teaching Hospital

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- Left breast UOQ area of asymmetry
- Left breast scattered macrocalcifications
- No spiculated masses or grouped microcalcifications
- Complementary ultrasound showed both breasts were of normal texture

- Left breast LOQ at 3-4 O'clock area of anterior paraxillary wall
- No internal vascularity on color Doppler

- Left breast para-areolar at 6 O'clock single ill-defined area of non-macular echogenic non-vascular content

- No cystic or solid masses are detected
- Normal skin thickness & nipple/areola complexes
- Right Non specific axillary lymph nodes
- Left axillary pathological lymph nodes: two nodes 3 mm and 4 mm largest measures 4x3.5 cm with irregular margins
- Level II&III pathological lymph nodes
- No evidence of supraclavicular lymph nodes

DIAGN

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1
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2
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RECOMME

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Conclusion:

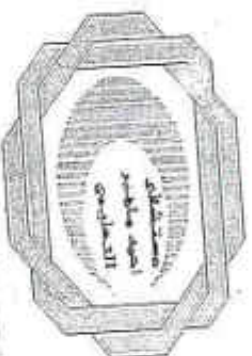
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General Organization for Teaching
Island Institutes
d Maher Teaching Hospital



الهيئة العامة للتعليمات والمناهج التعليمية

مستشفى أحمد ساهر التعليمي

السن : 49 سنة

2026) (1743

2026)

تاريخ الإصدار : 2026/07/20

اسم المريض : ورده عبد الودود عبد المحسن عبد العا

محول من :

تاريخ الحصول : 2026/07/07

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- 2) Sections examined revealed lymph nodal tissue infiltrated by the same tumor seen in the breast.

DIAGNOSIS :

- 1) Left breast mass, tru-cut biopsy; Invasive duct carcinoma, provisional core grade II. Intermediate grade DCIS, 10% (Solid pattern). Positive lymphovascular emboli.

- 2) Left axillary lymph nodes, tru-cut biopsy; Metastatic Invasive duct carcinoma to the lymph node.

RECOMENDATION :

Prof. Dr. Rasha Farouk, MD.

رقم البطاقة: 27702072500608



Date	17-8-2026	Age
Name	Warda abd el wadood	
Referred by		

2D and M mode measurements in mm				
parameter	28	Normal	parameter	Normal
Aortic root	26		IVS	0.8
Left atrium	28		PWT	0.7
LV EDD	39		Fractional Shortness	33
LV ESD	26		Ejection Fraction	62

Cardiac Valves :

- Mitral valve: Normal morphology
- Aortic valve: normal l aortic valve
- Tricuspid valve: Normal morphology.
- Pulmonary valve: Normal morphology and flow.

Comment

- Intact inter atrial and interventricular septi
- Normal doppler study
- good left ventricular systolic function with EF 62% by 2D and by visual estimation .
 - Left ventricle: normal LV internal dimensions.
 - Normal LV thickness and septum .
 - NRSWMA
- Right atrium: normal .
- Right ventricle: normal RV dimensions and normal function
- No thrombi or vegetations could be detected in LV.
- Normal pericardium NO pericardial effusion.

Conclusion:

Normal Echodoppler study

Much Obliged
Dr. A.ghazi

جمهورية مصر العربية
بطاقة حقوق الانتخابية



ورده
عبدالودود عبدالمحسن عبدالعاطي
١٩ اش احمد علي اسماعيل
بولاق الدكرور - الجيزة



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K03124675



٢٠٢٤/٠٥ ٢٧٧٠٢٠٧٢٥٠٠٦٠٨

رخصة منزل



أنتى مسلمة متروجة
أحمد عبدالوهاب محمد عبدالعاطى
البطاقة سارية حتى ٢٠٣١/٠٥/١٩





التاريخ: ٢٠٢٦/٧/٢٨

الاسم: وردة عبد الوادود

Ultrasonography of the Abdomen

Liver :
Size ☒ Normal ☐ Small ☐ Enlarged (☐ Mild - ☐ Moderate ☐ Marked)
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Gall Bladder :

Size ☒ Average ☐ Contracted ☐ Distended ☐ Removed
Wall Thickness : ☒ Normal ☐ Thickened
Mud ☒ No ☐
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Spleen :

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Kidneys :

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Focal lesion :	Cyst 33 mm	WAD
Stones :		
Back pressure :		

Pancreas : ☒ Normal ☐
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Aorta and para aortic nodes : Normal ☒

Urinary bladder ☒ No stones ☐
Prostate ☒ Normal ☐ Enlarged (vol. = cm³)

Impression : Bulky uterus with post. wall fibroid
47 x 37 mm

Signature

Dr. El

fatty liver
Rt Renal Simple Cyst
uterine fibroid



Name: Warda 3Abdelwadoud 45 Y, female

Date : 11/8/2026

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-Spleen : long axis :8.9cm , texture : homogenous , SP.V and collaterals : not dilated

-Kidneys : both are of normal position , texture : normal , back pressure : no , cysts : no , solid masses : no , stones : sandy , size : [11.6x4.7 cm , 11.3x4.3cm] .

-Pelvis:-UB: many internal echoes

- uterus & adenexa : a big heterogenous mass ? uterine versus adenexal : 5.1x5.0x5.2 cm for gynecological exam

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MANAL SHAKER



السن : 49 سنة

اسم المريض : وردة عبد الودود عبد المحسن عبد الو

محول من :

2026) (1743 - 2026 .)

تاريخ الإسلام : 2026/07/20

تاريخ الحضور : 2026/07/07

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RECOMMENDATION :

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محول من :
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(2026) (1743 - 2026)
تاريخ الحصول : 2026/07/20
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NATURE OF SPECIMEN :

One paraffin block (Left breast mass, tru-cut biopsy)

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One paraffin block: sections immunostained for ER, PR, Her-2/neu, Ki67, using supersensitive monoclonal antibodies with En-Vision TM+ (HRP) detection system employing peroxidase after antigen retrieval and blocking internal peroxidase and using DAB as chromogen using BenchMark GX automated immunostainer

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Progesterone receptors: Moderate nuclear staining in 50% of tumor cells.
Her-2/neu: No membranous staining of tumor cells.
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DIAGNOSIS :

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POSITIVE PROGESTERONE RECEPTORS, SCORE 6/8.
NULL HER-2/neu, SCORE 0
Ki67: POSITIVE IN 15% OF TUMOR CELLS.

RECOMENDATION :

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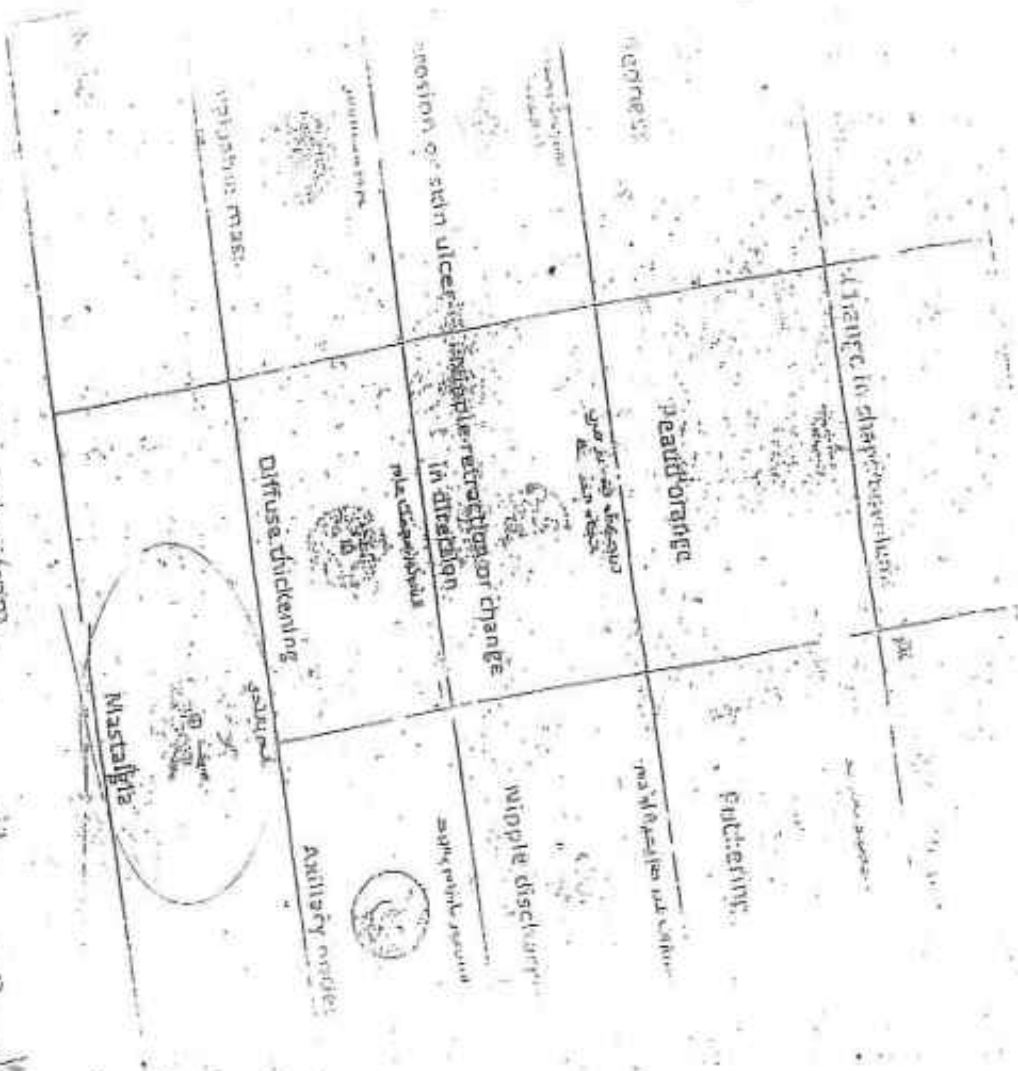
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St. Agilla. to Smo. N. 10.

to the individuals on pallets

Maximum glass-transition (mp)

120

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Conclusion

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Central and retroarticular

Ocella

Mapping study requested:

⑥ Sonography

Mammoscopy

NIRI broadcasts

128

[illegible]

Patient Name: *Warda Abdel Wadod Abdel-Mahssen*
Date: 1/7/2026

FULL FIELD DIGITAL MAMMOGRAPHY
COMPLEMENTARY ULTRASOUND

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- Scattered fibroglandular parenchyma. TAC of
- Left breast LOQ area of asymmetry.
- Left breast scattered macrocalcifications.
- No spiculated masses or grouped micro-calcifications.
- Complementary ultrasound showed both breasts are of homogeneous texture.
- Left breast LOQ at 3-4 O'clock area of altered parenchyma. TAC of
- No internal vascularity on color Doppler.
- Left breast para-areolar at 6 O'clock single tubular duct ectasia with nuclear echogenic non-vascular content.
- No cystic or solid masses are detected.
- Normal skin thickness & nipple/areola contour.
- Right Non specific axillary lymph nodes.
- Left axillary pathological lymph nodes have thin capsule. The largest hilum largest measures 4x3.5 cm with irregular margins.
- Level II&III pathological lymph nodes.
- No evidence of supraclavicular lymph nodes.

Conclusion:

- Left breast area of altered parenchyma with cystic duct ectasia. No lymph nodes for histopathological assessment. (BIRADS 4C).
- Left breast ductectasia.
- Right breast normal study (BIRADS 1).

Recommendations:

- Fine cut biopsy of left breast altered parenchyma with cystic duct ectasia.

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السن : 49 سنة

2026) (1743 - 2026)

تاريخ الإستلام : 2026/07/20

إسم المريض : وردة عبد الودود عبد المحسن عبد الله
محول من :

تاريخ الحضور : 2026/07/07

NATURE OF SPECIMEN :

Left breast mass, tru-cut biopsy.
Left axillary pathological lymph nodes, tru-cut biopsy

GROSS :

- Two specimens are received designated as:
1) Left breast: Multiple gray white tissue cores the longest measured 1.5cm long, totally submitted.
2) Left axillary lymph node: Multiple gray white tissue cores the longest measured 1.7cm long, totally submitted.

MICROSCOPY :

- 1) Sections examined revealed mammary tissue cores showing sheets, and clusters of malignant epithelial cells infiltrating among desmoplastic stroma.
Tumor cells show moderate pleomorphism and moderate tubule formation.
Intermediate grade DCIS, 10% (Solid pattern)
Positive lymphovascular emboli.

- 2) Sections examined revealed lymph nodal tissue infiltrated by the same tumor seen in the breast.

DIAGNOSIS :

- 1) Left breast mass, tru-cut biopsy;
Invasive duct carcinoma, provisional core grade II.
Intermediate grade DCIS, 10% (Solid pattern)
Positive lymphovascular emboli.

- 2) Left axillary lymph nodes, tru-cut biopsy;
Metastatic Invasive duct carcinoma to the lymph node.

RECOMMENDATION :

Prof. Dr. Rasha Farouk, MD.

رقم البطاقة: 27702072500608

Ahmad Maher Teaching Hospital

Patient Name: *Warda Abdel Wadod Abdel-Jelil*
Date: *1/7/2026*

FULL FIELD DIGITAL MAMMOGRAPHY COMPLEMENTARY ULTRASOUND

Indication:

- Left axillary palpable lump

Technique:

- CC and MLO views
- High resolution ultrasound

Findings:

- Scattered fibroglandular parenchyma (ACR B, U)
- Left breast UOQ area of asymmetry
- Left breast scattered macrocalcifications
- No spiculated masses or grouped microcalcifications
- Complementary ultrasound showed both breasts were of normal texture

- Left breast LOQ at 3-4 O'clock area of altered parenchyma
- No internal vascularity on color Doppler

- Left breast para-areolar at 6 O'clock single ill-defined hypoechoic nuclear echogenic non-vascular content

- No cystic or solid masses are detected
- Normal skin thickness & nipple/areola complexes
- Right Non specific axillary lymph nodes
- Left axillary pathological lymph nodes: two nodes 3 mm and 4 mm largest measures 4x3.5 cm with irregular margins
- Level II&III pathological lymph nodes
- No evidence of supraclavicular lymph nodes

DIAGN

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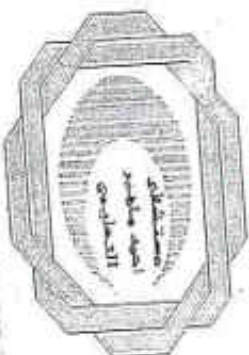
Conclusion:

- Left breast area of altered parenchyma with ill-defined hypoechoic nuclear echogenic non-vascular content
- Left breast ductectasia
- Right breast normal study (BI-RADS 1)

Recommendations:

- Fine cut biopsy of left breast altered parenchyma and hypoechoic nuclear echogenic non-vascular content

9



السن : 49 سنة

2026) (1743

اسم المريض : وردة عبد الودود عبد المحسن عبد العا
محول من :

تاريخ الحضور : 2026/07/07

تاريخ الإصدار : 2026/07/20

TURE OF SPECIMEN :

Left breast mass, tru-cut biopsy.
Left axillary pathological lymph nodes, tru-cut biopsy

FOSS :

Two specimens are received designated as:

- 1) Left breast: Multiple gray white tissue cores the longest measured 1.5cm long, totally submitted.
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1) Sections examined revealed mammary tissue cores showing sheets, and clusters of malignant epithelial cells infiltrating among desmoplastic stroma.
Tumor cells show moderate pleomorphism and moderate tubule formation.
Intermediate grade DCIS, 10% (Solid pattern)
Positive lymphovascular emboli.

2) Sections examined revealed lymph nodal tissue infiltrated by the same tumor seen in the breast.

DIAGNOSIS :

- 1) Left breast mass, tru-cut biopsy;
Invasive duct carcinoma, provisional core grade II.
Intermediate grade DCIS, 10% (Solid pattern)
Positive lymphovascular emboli.

- 2) Left axillary lymph nodes, tru-cut biopsy;
Metastatic Invasive duct carcinoma to the lymph node.

RECOMENDATION :

Prof. Dr. Rasha Farouk, MD.

رقم البطاقة: 27702072500608

Date : 05/07/2026
Name:
Referred By :

ورقة عمل الوردة

Lab. No : 260447
Sex : Female
Age : 48 Years

COMPLETE BLOOD COUNT

	Result	Ref. Range
Hemoglobin	12.3 g%	12 - 15
Red cell count	4.78 $\times 10^6 / \mu L$	3.8 - 4.8
Hematocrit (PCV)	36.9 %	36 - 46
MCV	77* fL	83 - 101
MCH	26* pg	27 - 32
MCHC	33 gm%	32 - 35
Platelet count	284 $\times 10^3 / \mu L$	150 - 410
White cell count	4.35 $\times 10^3 / \mu L$	4 - 11

Differential white cell count

	Result %	Normal	Result /mm	Normal
Neutrophils	49.2	37 - 78		2000 - 7600
Bands	1.0	0 - 6		
Segmented	48.2			
Lymphocytes	41.8	20 - 45		1500 - 3500
Monocytes	7.0	2 - 8		200 - 800
Eosinophils	2.0	1 - 6		40 - 400
Basophils	0	0 - 1		0 - 150

Laboratory director



٠١٠٦١١٣٥٠٧١ - ٠١١٢٥١٨٥٢٥٤

٦٧ امتداد عثمان محرم امام موقف الكونيسة - الدور الثالث



Date : 05/07/2026
Name:
Referred By :

وردة عبدالودود

Lab. No :
Sex : Female

260447
Age : 48 Years

Hematology Unit

Ref. Range

Prothrombin time :

Patient time	13.2	seconds	11 - 14
Control time	12.5	seconds	
Patient Concentration	85	%	70 - 100
INR	1.15		1 - 2
Ratio	1.1		



EGYPT LAB
Medical Laboratory

Laboratory director



٠١٠٦١١٢٥٠٧١ - ٠١١٢٥١٨٥٢٥٤

٦٧ امتداد عثمان محرم امام موقف الكونيسة - الدور الثالث



Lab. No :

Sex : Female

Age : 48 Years

معمل التحاليل الطبية
2609

Date : 11/08/2026

Name:

Referred By :

وردة عبد الوارد

COMPLETE BLOOD COUNT

		g%	Ref. Range
Hemoglobin	12.6	g%	12 - 15
Red cell count	4.8	$\times 10^6 / \mu L$	3.8 - 4.8
Hematocrit (PCV)	36.8	%	36 - 46
MCV	77*	fL	83 - 101
MCH	26*	pg	27 - 32
MCHC	34	gm%	32 - 35
Platelet count	263	$\times 10^3 / \mu L$	150 - 410
White cell count	4.56	$\times 10^3 / \mu L$	4 - 11

Differential white cell count

	Result %	Normal	Result /mm ³	Normal
Neutrophils	49.3	37 - 78		2000 - 7600
Bands	1	0 - 6		
Segmented	48.3			
Lymphocytes	42.7	20 - 45		1500 - 3500
Monocytes	5	2 - 8		200 - 800
Eosinophils	3	1 - 6		40 - 400
Basophils	0	0 - 1		0 - 150



Laboratory director

٠١٦١١٢٥٠٧١ - ٠١١٢٥١٨٥٢٥٣

٢٧ امتداد عثمان محرم أمام موقف الكونيسة. الدور الثالث



Lab. No :

260952

Sex : Female

Age : 48 Years

Date : 11/08/2026

Name:

فريدة عبدالوہد

Referred By :

Laboratory Report

		Ref. Range
Serum GOT (AST)	23.0 U/L	Up to 32 -
Serum GPT (ALT)	26.2 U/L	Up to 33 -
Serum Total Bilirubin	0.79 mg/dl	0.1 - 1.2
Serum Direct Bilirubin	0.16 mg/dl	Up to 0.25-
Indirect Bilirubin	0.63 mg/dl	-
Serum Sodium	138 mmol/L	136 - 145
Serum Potassium	3.95 mmol/L	3.5 - 5.1
Serum Urea	36.5 mg/dl	12 - 42
Blood Urea Nitrogen	17.06 mg/dl	6 - 20
S.Creatinine	1.13 mg/dL	0.5 - 1.4
Serum Uric Acid	5.8 mg/dl	2.6 - 6



Laboratory director

٠١٠٦١١٢٥٠٧١ - ٠١١٢٥١٨٥٢٥٤

٦٧ امتداد عثمان محرم أمام موقف الكونيسة، الدور الثالث



Lab. No.:

معمل التحاليل الطبية
26095

Date : 11/08/2026

ورقة عبدالودود

Sex : Female

Age : 48 Years

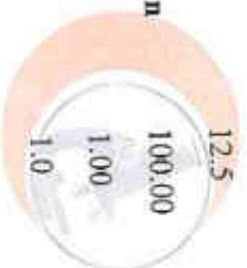
Referred By :

Hematology Unit

Ref. Range

Prothrombin time :

Patient time	12.5	seconds	11 - 14
Control time	12.5	seconds	
Patient Concentration	100.00	%	70 - 100
INR	1.00		1 - 2
Ratio	1.0		



EGYPT LAB

Medical Laboratory

Laboratory director



٠١٦١١٢٥٠٧١ - ٠١١٢٥١٨٥٢٥٤

٦٧ امتداد عثمان محرم أمام موقف الكونيسة ، الدور الثالث



Date : 11/08/2026

Lab. No :

د. ردة عبد الوارث

Sex : Female

Age : 48 Years

Referred By :

URINE ANALYSIS

Physical Examination:

Volume	Sample	ml.
Colour	Yellow	
Aspect	Clear	
Reaction (pH)	Acidic	
Sp. Gr	1.025	(N: 1.015 - 1.025)

Chemical Examination:

Albumin	Nil
Glucose	Nil
Acetone	Nil
Bilirubin	Nil
Bile Salts	Nil
Urobilinogen	Normal trace

Microscopic Examination:

Red cells	(4-6) /HPF
Pus cells	(2-3) /HPF
Epithelial Cells	(+)
Crystals	Ca oxalate (Few)
Amorphous	Free
Casts	Free
Parasites	Free
Others	Bacteria (+)
Comment	Blood :- (+)

Laboratory director



٠١٠٦١١٢٥٠٧١ - ٠١١٢٥١٨٥٢٥٤

٦٧ امتداد عشتان معمر أمام موقف الكونيسة . الدور الثالث



الهيئة العامة للعلاج والمستشفيات التعليمية
مستشفى أحمد ماهر التعليمي - مستشفى الجمهوريّة
وحدة الموجات الصوتية على القلب

Date	17-8-2026	Age	
Name	Warda abd el wadood		
Referred by			

2D and M mode measurements in mm				
parameter	28	Normal	parameter	Normal
Aortic root	26		IVS	0.8
Left atrium	28		PWT	0.7
LV EDD	39		Fractional Shortness	33
LV ESD	26		Ejection Fraction	62
				55-80%

Cardiac Valves :

- **Mitral valve:** Normal morphology
- **Aortic valve:** normal l aortic valve
- **Tricuspid valve:** Normal morphology.
- **Pulmonary valve:** Normal morphology and flow.

Comment

- Intact inter atrial and interventricular septi
- Normal doppler study
- good left ventricular systolic function with EF 62% by 2D and by visual estimation .
 - Left ventricle: normal LV internal dimensions.
 - Normal LV thickness and septum .
 - NRSWMA
- Right atrium: normal .
- Right ventricle: normal RV dimensions and normal function
- No thrombi or vegetations could be detected in LV.
- Normal pericardium NO pericardial effusion.

Conclusion:

Normal Echodoppler study

Much Obliged,
Dr. Aghazi